NCCP 00226 · VERSION 9

Weekly paclitaxel · relapsed or refractory SCLC

Paclitaxel 80 mg/m² is infused on Days 1, 8, 15 and 22 of a 28-day cycle. The consultant may instead prescribe Days 1, 8 and 15. Each planned weekly dose needs review. Continue while helping and tolerated; NCCP does not set a fixed cycle cap.

Source reconciled; independent clinical, pharmacy and patient/PPI review pending. Selected effects; not a complete medicine leaflet.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 Hands / feet

Paclitaxel

Report numbness, weakness or changes in walking and hand use before the next dose.

3 Hair / scalp

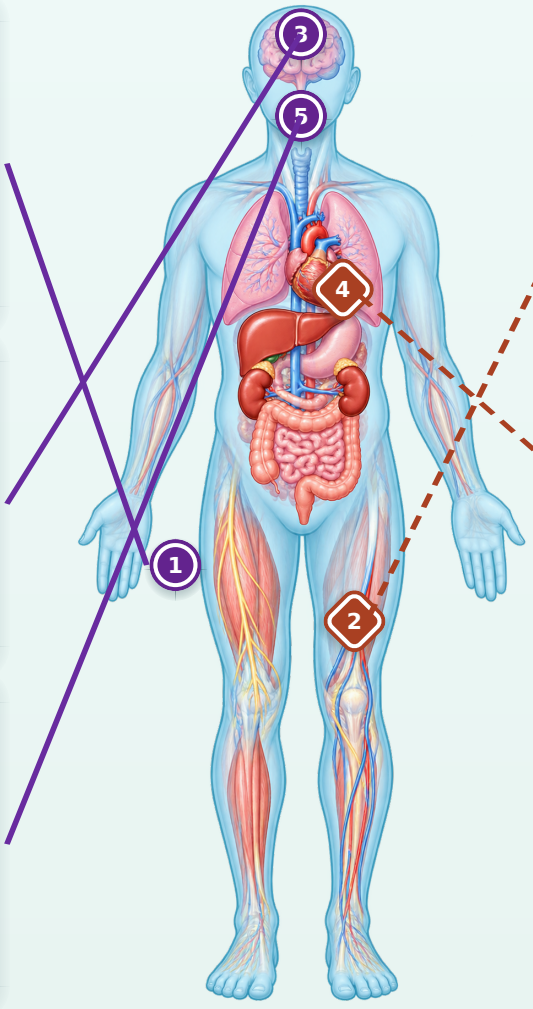
Paclitaxel

Hair loss can occur; ask about support before treatment.

5 Mouth

Paclitaxel

Report painful ulcers, difficulty drinking or poor intake.



2 Blood / infection

Paclitaxel

Fever, shivers or sudden illness need immediate contact.

4 Breathing

Paclitaxel

New or worsening breathlessness or cough needs urgent assessment.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

The body map highlights symptoms to report. It does not show where your cancer is or predict which effects will occur.

> Call the oncology contact number immediately for fever, shivers or sudden illness, severe or new breathing difficulty, chest pain, fainting, severe abdominal pain or persistent diarrhoea. Tell staff immediately about infusion symptoms. Report new numbness, weakness, painful skin changes, swelling or visual changes promptly; do not wait for the next cycle.

Your treatment, at home.

Important medicine-specific effects

Paclitaxel

- Low blood counts / infection
- Sensory or motor neuropathy, potentially persistent
- Hypersensitivity / infusion reaction
- Hair loss and nail / skin changes
- Muscle and joint aches
- Fatigue and mouth / bowel symptoms
- Rare serious lung or cardiac effects
- Serious infection / sepsis
- Rare serious bowel inflammation; vision changes

Surgery, radiation and monitoring: open the whole-treatment map or scan the QR.

Make the plan practical

Premedication

Premedication reduces infusion-reaction risk. The first-dose and later-dose plans may differ under the NCCP weekly pathway. The team must reconcile your exact product and reaction history; do not omit premedication yourself.

Nausea, bowel symptoms and pain

Use your individual anti-sickness and pain plan. Contact the team if symptoms are uncontrolled or you cannot drink. Antihistamines and other support medicines may cause drowsiness; arrange transport if advised.

Nerves and daily activities

Report tingling, numbness or weakness early, including effects on buttons, writing, walking or balance. Protect hands and feet and take care with hot surfaces if sensation changes.

Infection and blood counts

Each treatment needs its own blood and symptom review. A growth-factor injection may be prescribed after individual risk assessment; follow its separate instructions. Fever or sudden illness needs immediate contact on any day.

Emergency: 112 / 999. Severe breathing difficulty, collapse, seizure or severe chest pain needs emergency help.

Contact oncology immediately for temperature ≥37.5°C or ≤36°C, shivering or suddenly feeling unwell, even with a normal temperature. Follow your own hospital's instructions. If you cannot reach the team, seek urgent medical assessment.

YOUR ONCOLOGY TEAM · DAYTIME

Insert numbers here

Acute Oncology Service

YOUR ONCOLOGY TEAM · OUT OF HOURS

Insert numbers here

Ask for medical oncology

EVENINGS / OVERNIGHT

Your out-of-hours pathway / nearest ED

Other hospitals: use the numbers your team provides.

Complete reference, visual timeline and passport

https://sactcheck.com/patient/00226/lung/

Sources and review status are available online. Follow your own prescription and hospital instructions.